Annotator Guide — transcribing an encounter in ELAN

Step-by-step instructions for producing one gold transcript.

This guide tells you **what to do**. For *why* any rule exists, see transcription_standards.md — cited as **[S§n]** throughout. When this guide and the standards disagree, the standards win; tell whoever maintains them.

> **You work blind.** No machine transcript, no other annotator's file, no existing draft. If someone offers you one, decline — it biases the result in a way that can't be detected afterward. **[S§9]**

Before you start

You need:

- **ELAN 7.x** installed
- `elan/template.etf` — the project template
- Your assigned case's **video** (`.mp4`) and **audio** (`.wav`)
- **Headphones.** Not laptop speakers. You will be distinguishing `uh-huh` from `uh-uh`, and that decision changes a patient's answer from yes to no.
- The **cheat sheet** at the bottom of this page, printed or on a second screen

Expect roughly **6–10× real time** for a first pass. A 20-minute encounter is most of a day. That's normal and it is not a sign you're doing it wrong.

Step 1 — Create the file from the template

File → New.

In the dialog:

1. Add **both** media files — the `.mp4` *and* the `.wav`. The video shows you who is speaking; the `.wav` gives you a real waveform instead of a video-derived guess.
2. Select `elan/template.etf` as the template.
3. OK.

You should see six empty tiers: `LEARNER`, `PATIENT`, `PRECEPTOR`, `ANNOUNCEMENT`, `OUTSIDE_ROOM`, `NOTES`. **[S§4]**

Save immediately as `<case_id>.pass1.eaf` in the case's gold folder. Save often after that.

> **Never create a tier** unless someone genuinely present fits none of the five speaker tiers. If that happens, stop and ask — it usually means something about the case is unusual and worth recording.

Step 2 — Listen through once, before annotating anything

Play the encounter start to finish at normal speed. Don't type.

You are learning: how many people speak, what each voice sounds like, where the hard patches are, and whether anything odd happens (equipment noise, an interruption, hallway bleed).

This costs 20 minutes and saves hours. Voice identification gets dramatically easier once you know who's in the room.

Jot anything notable straight into `NOTES` as you go.

Step 3 — Segment, one tier at a time

Switch to **Segmentation Mode** (the tab across the top).

Work one speaker tier at a time, through the whole recording. Select `LEARNER`, pass through the encounter marking only where the learner speaks. Then `PATIENT`. Then `PRECEPTOR`. Then the others if they have any content.

This feels like more listening than doing all speakers at once. It is faster and more accurate, for two reasons:

- "When does *this one person* speak?" is a far easier perceptual task than tracking everyone simultaneously.
- **Overlapping speech comes out correct for free.** Two people talking at once is just two tiers with annotations at the same time. You never mark overlap by hand. **[S§4]**

What counts as one segment

One utterance or breath group — a natural phrase boundary, usually a few seconds. *Not* the whole turn. **[S§5]**

If the learner talks for 90 seconds, that's many segments, not one. Long annotations are miserable to type and to check.

Turn-level boundaries for the diarization reference get rebuilt automatically at export. You don't need to think about them.

How precise?

Bound the speech; don't chase it. Roughly a tenth of a second is plenty. Nothing downstream needs better.

Don't mark pauses. Silence between segments is already measured by the timestamps. **[S§5]**

You can **set "delayed mode"** if you find it helpful: it will automatically place a boundary a number of milliseconds before you actually hit . 200 ms might be a good place to experiment depending on your reaction time.

Shortcuts

- The default configuration is to hit **Enter** to mark the beginning and again at the end of a segment.

- The up-arrow and down-arrow keys will change the tier you are segmenting
- You can drag a segment around in the area above the tiers. If it's selected (green), you can delete with the **Delete** (or **Backspace**) key and pull its edges back and forth. But don't feel you have to unless you think it'll be difficult to transcribe later. (The transcription mode automatically plays the segment for you)
- **Control-space** starts and stops the player
- **space** will play only the sound in the highlighted (blue) part of the audio

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Step 4 — Transcribe

Switch to **Transcription Mode**. It shows your segments in a column and plays each one as you land on it, looping while you type. This is much faster than typing in the timeline.

Work tier by tier, same order as Step 3.

The rules while typing

Type exactly what you hear. Every false start, repetition, and grammatical error stays. Do not tidy. The clean version is generated later — cleaning at the keyboard cannot be undone. **[S§6]**

Type real names as you hear them. "Hi, I'm Jamie" is transcribed as *Hi I'm Jamie*. Do **not** write `[LEARNER_NAME]`. Redaction happens mechanically later, and doing it by hand creates errors and scores against providers that were correct. **[S§8]**

Loop the segment as many times as you need. Three or four passes on a hard one is normal.

Work in ~30-second bites

Segment about 30 seconds, transcribe it, then move on. Don't segment the entire recording and then transcribe the entire recording — you'll lose your sense of the audio and rework more.

Nothing in the file records these bites; it's just how to pace the work.

Step 5 — When you're not sure

Never guess. A flagged unknown is recoverable; a confident error is not.

Situation	Write
Can't make out any of it	`[unintelligible]`
Have a decent guess	`[unintelligible: pressure?]`
Can't tell `uh-huh` from `uh-uh`	`[unintelligible: uh-huh?]`
Don't know the medical term	best phonetic guess in `[unintelligible: ...?]`, then flag it

Uncertain spans are excluded from scoring on both sides, so marking one costs nothing. Guessing wrong costs a lot. **[S§6]**

The one that matters most

`uh-huh` means yes. `uh-uh` means no. One letter apart, opposite meanings, and a patient's answer to a clinical question can hinge on it.

- `uh-uh` (no) — glottal catch between the syllables, pitch falls then rises
- `uh-huh` (yes) — audible /h/, pitch rises

If you can't tell, listen to what happens next. The other speaker's response almost always settles it. Still unsure — mark it uncertain. **[S§6]**

Step 6 — Use the NOTES tier

Drop a note whenever something affects transcription quality: a noise burst, a very quiet stretch, heavy crosstalk, an unfamiliar accent, a mic problem, an interruption.

Free text, rough timing, **not exhaustive**. This is how anyone later explains why a case scored the way it did. **[S§4]**

Don't use NOTES for transcription content.

Step 7 — Finish and hand off

1. **Check every tier.** Any segment still empty? Either transcribe it or delete it — empty segments become phantom turns in the diarization reference.
2. **Spot-check five random segments** against the audio.
3. **Save** as `<case_id>.pass1.eaf`.
4. **Tell the project lead it's done.**

Once you report it complete, **pass 1 is frozen**. Don't reopen it to fix something you thought of later — report the concern instead. It gets handled in adjudication, where the change is recorded. **[S§9]**

> **File handling:** your `.eaf` contains real names. Keep it on managed project storage. Never email it, never put it in a personal cloud folder, never commit it. **[S§8]**

Cheat sheet

Filled pauses — the only three

```
uh      um      hmm
```

Not `er`, not `erm`, not `ah`. **[S§6]**

Backchannels — write them, they mean something

```
mm-hmm   yes        uh-uh   no
uh-huh   yes        mm-mm   no
```

```
huh?     "say that again?"
```

Marks

```
so:                stretched sound (one colon, however long it is)
par-               cut-off word or false start
I I I went         repetition — just type it, no markup
[unintelligible]           couldn't hear it
[unintelligible: word?]    best guess
```

Optional, when it's interesting

```
[laugh]   [cough]   [sigh]   [throat clear]
```

Numbers — as spoken

```
✅ one twenty over eighty        ❌ 120/80
✅ fifteen milligrams            ❌ 15 mg
```

Never

```
❌ [LEARNER_NAME]      — type the real name
❌ overlap markers     — separate tiers handle it
❌ pause marks         — timestamps handle it
❌ tidying grammar     — verbatim means verbatim
❌ guessing            — mark it uncertain
```

Domain terms

Spell from `benchmarks/configs/glossary.yaml`. Term missing? **Add it** — that's how the list grows.

Common mistakes

Mistake	Why it hurts
Segmenting all speakers in one pass	Much harder, and you lose overlap for free
One segment per whole turn	Painful to type, painful to review
Cleaning up speech while typing	Irreversible; the clean version is generated
Writing `[LEARNER_NAME]`	Breaks scoring and hides what was actually said

Mistake	Why it hurts
Guessing a term instead of flagging	Turns a free non-answer into a counted error
Chasing exact word boundaries	Nothing uses that precision
Reopening a frozen pass 1	Destroys the blind-pass property

Questions

Anything this guide doesn't cover, ask rather than improvise — an improvised convention applied across a whole case is expensive to undo, and usually means the standards need a fix that helps every annotator. **[S§12]**